Intervention and Practice: high-yield review (about 32% of the exam)

Process and engagement

- Engage, assess, plan, intervene, evaluate, terminate. Engagement and consent come before assessment with mandated clients.
- **Motivational interviewing:** partnership, acceptance, compassion, empowerment; OARS (open questions, affirmations, reflections, summaries); evoke change talk; respond to discord without arguing.
- **Stages of change:** precontemplation, contemplation, preparation, action, maintenance. Match the intervention to the stage.
- Missed sessions? Ask about practical barriers before labeling resistance.

Crisis, trauma, harm reduction

- **Roberts' 7 stages:** assess lethality and safety, build rapport, identify the problems, deal with feelings, explore alternatives, make an action plan, follow up.
- **Trauma-informed principles (SAMHSA):** safety; trustworthiness and transparency; peer support; collaboration and mutuality; empowerment, voice and choice; cultural, historical and gender issues. Realize, recognize, respond, resist retraumatization.
- **Harm reduction:** meet people where they are (naloxone, safer use, avoiding drunk driving). Not "condoning use."

Match the model

Model	Signature moves
CBT	Identify and test automatic thoughts; behavioral experiments; exposure hierarchies
DBT	Mindfulness, distress tolerance, emotion regulation, interpersonal effectiveness
Solution-focused	Miracle, exception, scaling and coping questions; small positive goals
Narrative	Externalize the problem; unique outcomes; re-author the story
Psychodynamic	Insight, transference; ego-supportive work for fragile or crisis clients
Person-centered	Empathy, unconditional positive regard, congruence
Task-centered	Brief, time-limited; client-chosen tasks toward specific problems

Family models

Model	Cues
Bowen	Differentiation, triangles, cutoff, genogram, calm coach, I-position
Structural (Minuchin)	Boundaries (enmeshed, disengaged), hierarchy, coalitions; joining, enactment, boundary making, unbalancing
Strategic (Haley, Madanes) and MRI	Directives, paradox, ordeals; the attempted solution is the problem; reframing
Milan	Hypothesizing, circular questions, neutrality, positive connotation
Experiential (Satir, Whitaker)	Communication stances, sculpting; use of self
EFT / Gottman	Pursue-withdraw cycle, attachment / Four Horsemen (contempt worst) and antidotes

Partner violence: screen individually; no joint sessions with coercive control or fear. No paradox in crises.

Groups

- **Stages:** forming, storming, norming, performing, adjourning (Boston model: preaffiliation, power and control, intimacy, differentiation, separation). Conflict in early sessions is normal.
- Leader is more active early, less central later. Screen and prepare members; set norms, including confidentiality limits.
- **Yalom's factors:** hope, universality, information, altruism, family recapitulation, socializing, imitation, interpersonal learning, cohesiveness, catharsis, existential factors.
- Monopolizer or scapegoating: handle in the group first, protect members; meet privately only if needed. Never shame a member.

Evaluating practice

- EBP = research + expertise + client values. AB design shows change, not cause; multiple baseline shows cause without withdrawal; ABAB is strongest but withdraws treatment.
- Reliability (consistency) is needed for validity (accuracy), not the reverse.
- Needs assessment (before) · formative/process (during, fidelity) · summative/outcome (after) · cost-effectiveness (cost per outcome) vs. cost-benefit (all in dollars). Outputs count services; outcomes measure change.

Supervision and administration

- Kadushin: administrative, educational, supportive. Supervision has authority and shared accountability; consultation is advisory.
- Parallel process: client-worker dynamics replay in supervision. Supervision is not therapy.
- Burnout (exhaustion, depersonalization) vs. secondary traumatic stress (trauma symptoms) vs. vicarious trauma (changed worldview).
- Budgets: line-item (categories), functional (cost per unit of service), program (cost per outcome). Allocate scarce resources by fair, consistent rules.

Termination

Plan it from the start; review gains, process feelings, plan maintenance and referrals; in groups, announce the ending early.

Sources: Miller & Rollnick (2023); Prochaska & DiClemente (1983); Roberts (2005); SAMHSA (2014) SMA14-4884; Linehan (2015); Minuchin (1974); Bowen (1978); Yalom & Leszcz (2020); Bloom, Fischer & Orme (2009); Kadushin & Harkness (2014); Kettner, Moroney & Martin (2022).